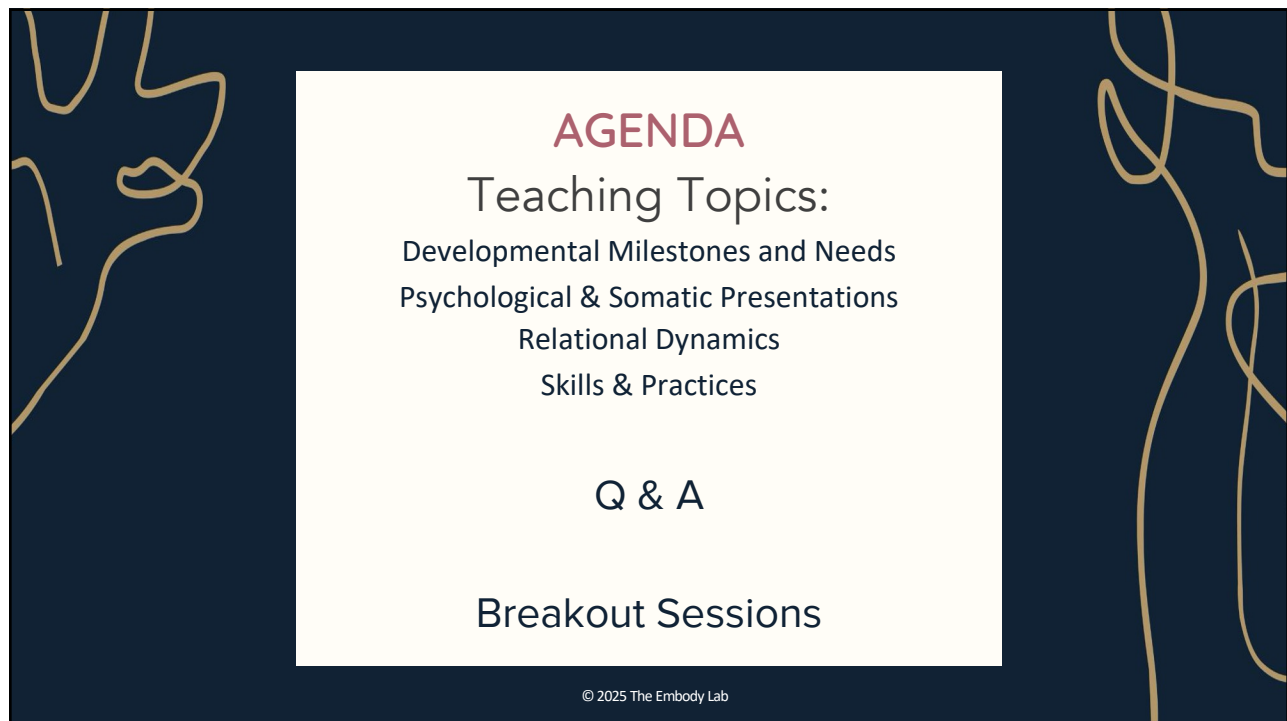




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Arriving Practice

- Embody your Boundaries
 - Stand your Ground
 - Find your core
 - Wall Push
 - Power Poses

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SECTION TITLES

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Developmental Milestones & Needs

- Increased independence while sustaining a sense of connection in relationship
- As a toddler learns to walk and become increasingly mobile, a new set of needs arise having to do with willfulness, autonomy, and sustained connection. This stage begins some time around 18 months to 2 years.
- Basic needs:
 - To be seen as a separate self without fear of losing a sense of connection to caregivers.
 - To know “It is okay to be me.”
 - To feel protected, but not overprotected or restricted, while exploring the world.
 - To have feelings and sensations reflected and attuned to by caregivers who also guide healthy expression of big feelings such as anger, frustration, or normal and healthy “push-pull” dynamics.

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Development Trauma during this Phase

Significant threats during this stage of development include:

- **Any abuse**
- **Any neglect**
- **Any chronic lack of emotional safety**
- **Any chronic boundary violation**
- **Shaming of the child around toilet training**

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Development Trauma during this Phase

Significant threats during this stage of development include:

- Chronic power struggles with at least one parent
- Love is conditional and based on the child's performance
- The parent discounts or dismisses the child's needs.
- The parent is overprotected which restricts the child's ability to develop autonomy or an independent sense of self as they explore their environment
- The child is expected to grow up too quickly which leads to premature independence (pushed out of dependency)
- Any significant loss or trauma that is not attended to by caregivers

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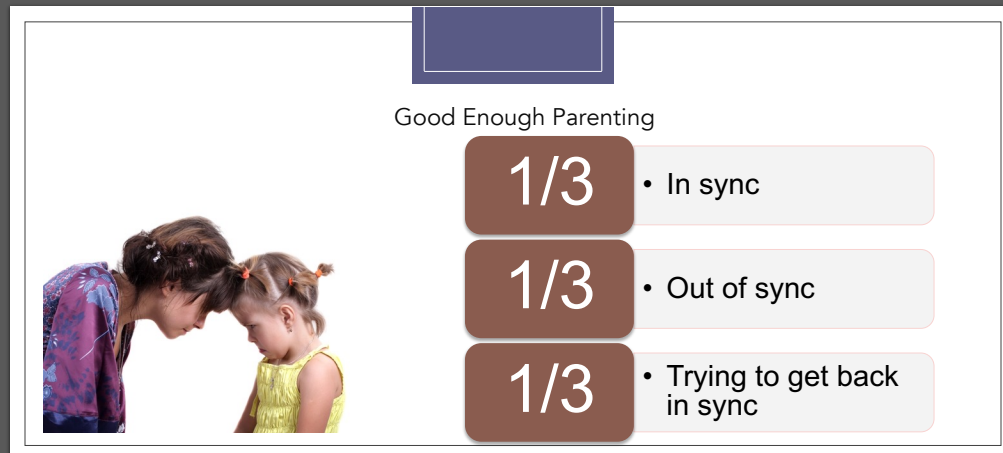
3 Forms of Childhood Stress (Harvard Center for the Developing Child)

- **Positive stress response:** Is temporary and resolves into a positive connection to loving caregivers. Tones the nervous system to cope with stressful situations in adulthood.
- **Tolerable stress response:** A child faces a single-incident trauma. Loving caregivers in a supportive environment help the child process their experience. Associated with the development of positive coping skills to handle stress in adulthood.
- **Toxic stress response:** Profound or ongoing traumatic events take place without support from loving adults. Associated with a negative outcome on emotional and physical health in adulthood.

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Rule of Thirds (Tronick, 2007)



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Rupture and Repair

- Errors in attunement followed by repair serve a purpose.
- Help the child handle the inevitable stresses of life—and trust that they can resolve into positive affect states (Tronick, 2007).
- Repair:
 - Recognize the rupture
 - Attune to the child
 - Stay Engaged until reconnection is achieved
- Over time, an infant internalizes the mother's influence, learns self-regulation, and learns to tolerate internal distress.



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Relational Trauma

- In the case of relational trauma, caregiver behaviors are dysregulating for the infant.
- Caregivers who are dysregulated themselves are less available for repair behaviors which leaves the infant in intensely disrupted states for extended periods of time.



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Core Needs

- Core needs:
 - It is okay to be me
 - I can explore the world in safety
 - I can be in relationship with the world and in support with my caregiver.
 - I am powerful and independent and have preferences.

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Negative Cognitions

- **Negative Cognitions:**
 - I must perform to be loved
 - It is not okay to be me
 - I cannot protect myself
 - I am unlovable
 - I am not supported in my creativity
 - I will be dominated when I exert needs
 - I can't disagree or I lose connection
 - I have preferences that I have to resist to not disappear

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Somatic Presentations

- **Fawning/Over-Adaptation:** forward-leaning, collapsed chest, shallow breath
- **Chronic Defiance:** rigid jaw, bracing shoulders, narrowed focus
- **Avoidant Independence:** disconnected core, floating energy, lack of grounding
- **Collapse/Indecision:** flaccid tone, unfocused eyes, low voice
- **Freeze in Choice:** stillness without alertness, stiff extremities
- **Flight:** excessive anxiety that is not regulated by caregiver, client stays stuck in anxious loops, high motor expressions such as running or fidgeting.

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Understanding Shame

- **Shame: the thwarting of positive emotions. An experience of rejection when we reach out for connection. Like when a child expresses excitement to a parent who responds with disinterest, shame occurs when our joy goes unmatched, when our gifts are not received, or when we take a risk and experience failure.**
- **Shame is an interpersonal emotion and shows up as embarrassment, humiliation, and shyness. Shame causes us to blush, look away, hide our face, and collapse our posture. It is an act of turning away from something deeply desired.**
- **Shame can arise because of childhood abuse or neglect. However, shame is also the result of an accumulation of small and subtle rejections of your authentic self-expression.**
- **Shame has an evolutionary purpose. This emotion holds the remnants of our unextinguished longings.**

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Working with Shame

- **Regulate Yourself: Notice any tendency to shutdown with the client.**
- **Relational attunement and validation.**
- **Differentiate "I am bad" from "I feel bad"**
- **Return responsibility to appropriate part "you were just a child; they were the adults"**
- **Access underlying legitimate anger**
- **Build capacity for self-compassion and acceptance**
- **Build boundary awareness**
- **Enhance a felt sense of empowerment**
- **Parts work (differentiate from and compassionately attend to the part carrying the shame)**
- **Attend to underlying grief**

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Transference & Counter-transference

- **Subjective Countertransference:** Evoked experience parallels is related to material from your own life.
- **Objective Countertransference:** Evoked experience provides insight into the inner world of the client.
- **Somatic Resonance:** Somatic reactions that a therapist feels in response to a client (Keleman, 1987)
- **Mindful reflection:** Allows the therapist to differentiate between subjective and objective countertransference. (Often, it's both)
- **Once you feel it, it is your responsible to attend to the sensations with self-care.**

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Relational Re-Enactments

- **Re-Enactment:** We all recreate relationship patterns that match what we know and expect. These dynamics are an inevitable part of therapy (Bromberg)
- **Mutual Regression (Schoore):** Both therapist and client will confront our own relational history.
- **Rupture and Repair:** Helps clients form new relationship expectations:
 - Recognize the rupture
 - Attune to the Client
 - Stay Engaged until reconnection is achieved
- **The mindful therapist provides a safe space to help clients nonjudgmentally reflect upon relational patterns so that change can occur.**

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Skill: Relational Resourcing

- **"Notice how it feels as you hear these words:**
 - I am right here with you
 - I am listening to you
 - I am here to support you
 - I am giving you as much space as you need
 - I wish that I could have protected you back then...
- **There is no "right" or "wrong" response to this inquiry. Allow yourself to notice whatever arises with curiosity.**
- **Is there relief or a positive emotional response? If so, let yourself receive this fully.**
- **Is there discomfort or disbelief? If so, how is this feeling familiar.**
- **Know, that this pain is coming to the surface of your awareness to be attended to and resolved.**



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Skill: Relational Repair in therapy

- **Acknowledge the disconnection:** e.g., "I notice that you became quiet and looked away." "I noticed you stopped speaking." "You seemed quiet after I said..."
- **Offer understanding and express willingness to learn more:** "I think I missed something when..." (Ask if you have an accurate understanding about what happened. Let them know you are interested in their perspective.)
- **Reflect their experience nondefensively:** "You're right, I did..." or "I hear that you felt..." (It might be helpful to explain your reasoning for your behavior; however, be sure that the explanation does not feel as though you are making an excuse or minimizing the client's experience. You might also offer an apology to the client. In doing so, invite the client to notice how it felt to receive an apology as this may provide contrast to events in their past which were never repaired. Stay with the process until repair is successful.)
- **Appreciate the resolution:** "How do you feel about this conversation?" Recognize their willingness to stay engaged with the process, "Thank you for telling me how you felt. I feel closer to you now." Acknowledge the courage that it takes to stay with discomfort, invite them to notice how they went from disconnection back into an experience of connection.

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Three Diaphragms

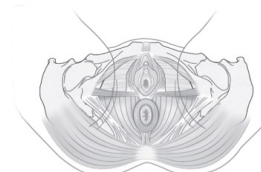
- The Laryngeal Diaphragm at the throat
- The Respiratory Diaphragm at the lower ribcage
- The Pelvic Diaphragm at the root of your body
- The 3 diaphragms are in constant movement and none work in isolation. Together, their intersectional action provides us with the key to both internal and external bio-psycho-social stability.



1 Thoracic diaphragm



2 Respiratory diaphragm



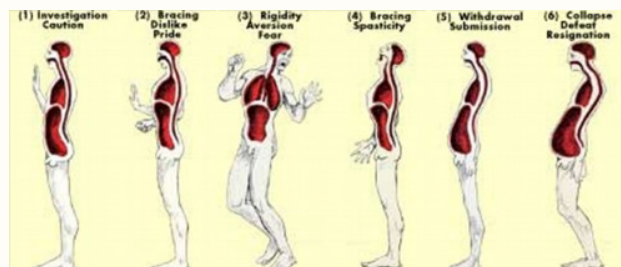
3 Pelvic diaphragm

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Emotional Anatomy

- Diaphragms of the body & holding patterns (character strategies):
 - Palette in the throat
 - Diaphragm at the ribcage
 - Pelvic floor
- Rigid (Hypertension)
- Disengaged (Hypotension)



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Energetic Containment

- **Bandhas & Containment**

- **Mula Bandha (Root Lock):** Even without directly practicing the advanced form of the *bandhas*, you can still indirectly integrate the concept of containing your energy.
- **Uddiyana Bandha (Upward Abdominal Lock):** exhaling completely and drawing the navel and diaphragm inward and upward while holding the breath out. Beneficial for clearing stagnation from the digestive organs and lungs.
- **Jalandhara Bandha (Chin Lock):** drawing the chin slightly downward and back. the lid of the energetic container that helps you to gather a potent felt sense of your presence.

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Somatic Practice

- **Dirgha Pranayama**
- **Stair Breathing with Breath Holds:**
 - **Top of Inhalation**
 - **At end of Exhalation**

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Discussion Prompts

1. Talk about the Developmental Milestones related to the Independence in Relationship Stage. How have you noticed these themes show up in your work (or personal development).
2. In what ways have themes of rupture and repair shown up in your work. How does this bring up personal history for you in the form of countertransference? What support systems do you have in place to help you mindfully differentiate between subjective and objective countertransference?
3. What arose for you in the energetic containment practice? Was it easy or difficult for you to engage the three diaphragms/bhandas? What subtle or not so subtle shifts are you aware of in your body?

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